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February 23, 2026

Submitted Electronically

The Honorable Robert F. Kennedy, Jr., JD, LLM
Secretary
U.S. Department of Health and Human Services
200 Independence Avenue SW
Washington, DC 20201

RE: HHS Health Sector AI RFI (RIN 0955-AA13)

On behalf of the more than 10,000 physiatrists of the American Academy of Physical Medicine and Rehabilitation (AAPM&R), we appreciate the opportunity to submit comments in response to the Department of Health and Human Services Request for Information: Accelerating the Adoption and Use of Artificial Intelligence (AI) as Part of Clinical Care. AAPM&R is the national medical specialty organization representing physicians who are specialists in physical medicine and rehabilitation (PM&R). PM&R physicians, also known as physiatrists, treat a wide variety of medical conditions affecting the brain, spinal cord, nerves, bones, joints, ligaments, muscles, and tendons. PM&R physicians evaluate and treat injuries, illnesses, and disability and are experts in designing comprehensive, patient-centered treatment plans. Physiatrists utilize cutting-edge as well as time-tested treatments to maximize function and quality of life.

AAPM&R agrees that the Department of Health and Human Services (HHS) has a significant role in shaping the environment in which AI is used in healthcare. As such, we are grateful for the opportunity to provide feedback on barriers in regulation and reimbursement associated with AI implementation. Our Academy and our membership have been exploring the role of AI in clinical care as well as the role it can play in administrative tasks associated with practicing medicine. We are encouraged by the opportunities available and the potential positive impact on patient health outcomes and physician wellbeing. However, we urge caution with respect to rapid implementation of AI tools without clear guardrails. Though we recognize the need to be nimble as technologies are evolving quickly, advancements in healthcare require adequate infrastructure, regulation, and reimbursement.

Regulation

Current regulatory categories do not neatly fit AI tools in rehabilitation. This can lead to clinician hesitation to adopt tools without clear expectations for privacy, safety, validation, transparency, and liability. To address this limitation, greater

clarity is needed to provide clinicians certainty regarding their risks and responsibilities and to facilitate greater comfort with the adoption of new technologies.

AAPM&R recommends HHS explore regulation around “black box” technologies. Specifically, without sharing proprietary information or protected health information, AI companies should be required to make their algorithms and data sources transparent and include physicians in the development of algorithms. This will help move the needle toward creating trust and understanding. Psychiatrists are especially concerned that “black box” AI may underrepresent pertinent information from persons with disabilities. This lack of trust leads to hesitation around adoption. Transparency in algorithms and physician involvement may lead to more confident adoption and widespread use of AI and facilitate partnerships between AI companies and the physician community to better address the needs of certain populations such as persons with disabilities.

AAPM&R also underscores the importance of physician judgment in clinical care, and the need to ensure that a physician – in collaboration with the patient - is the ultimate decision maker. We recommend that HHS establish requirements that physician oversight is required when AI is used for treatment decisions. Likewise, we recommend that physician review be required when AI is used in making insurance or prior authorization determinations, in order to ensure necessary human oversight.

AAPM&R also recommends HHS review HIPAA regulations to consider the need for updates related to privacy and security risks associated with AI and other new technology innovations. At a minimum, physicians and the medical community should be provided with modernized guidance around HIPAA compliance related to AI use in research, administrative tasks, and clinical care. Absent clear, up-to-date guidance, physicians are hesitant to implement cutting edge technologies.

Reimbursement

For physician practices, cost or affordability of AI continues to be a significant barrier to implementation of AI. While this applies to all practices, cost can be particularly burdensome for small group practices, private practices, solo practitioners, and rural practices. AI technologies require significant investment of both time and financial resources without a guaranteed return on investment. AAPM&R recommends thoughtful exploration of reimbursement policy with respect to the range of AI technologies, including in the Medicare program.

Current reimbursement for AI-supported care is limited due to a lack of straightforward connection between AI and the current fee-for-service payment structure. We recognize that the American Medical Association’s (AMA’s) Current Procedural Terminology (CPT) Editorial Panel is working to modernize the CPT code set to accommodate the range of rapidly emerging technologies including AI-enabled care. AAPM&R recommends HHS and the Centers for Medicare and Medicaid Services (CMS) support this development of new coding pathways for digital healthcare. Further, when it comes to reimbursement for AI tools, we are concerned that paying for high-cost AI equipment under the current Medicare physician fee schedule (MPFS) may become unsustainable due to budget neutrality rules. We urge HHS and CMS to



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explore mechanisms for paying for AI equipment that do not negatively impact payment for physician services under the MPFS.

Beyond fee-for-service, AI tools are likely also to be an asset in alternative payment models (APMs). We are concerned, however, that AI is being used in APMs without the guidance and input from the physician community. We strongly recommend that clinical experts are at the table when discussing incorporation of AI into APMs – including in the model conceptualization and development phases. Furthermore, to appropriately motivate physician use of AI in APMs, HHS should work with the CMS Innovation Center to appropriately incorporate AI costs into model benchmarks and spending calculations. If these costs are not properly captured, physicians and hospitals will struggle to invest in new technologies necessary to succeed under new models.

Conclusion

Thank you for your consideration of our comments. If you have any questions or would like more information regarding any of the recommendations offered above, please contact Carolyn Millett, Senior Director of Practice, Reimbursement, and Regulatory Affairs at cmillett@aapmr.org or (847) 737-6024.

Sincerely,

A handwritten signature in black ink that reads 'Susan Hubbell'.

Susan Hubbell, MD, FAAPMR
Chair, Health Policy, Practice, and Advocacy Committee